



MC - 6652

Name : Mrs. D SHARADHA 15786

Age/Gender : 42 Years / Female

Ref.By : DR Y SUMAN KUMAR

Req No. : SMN2681847

Sample Type : WB-EDTA

Client Name : NEO CARE SPECIALITY-CMLTSD23

Vial ID : 3238252

Collected On : 17-May-2026 02:42 PM

Registered On : 17-May-2026 02:38 PM

Reported On : 17-May-2026 03:43 PM

Client Code : CMLTSD23

Glycosylated Hemoglobin (GHb/HbA1c)

Test Name	Observed Values	Units	Biological Reference Intervals
* GLYCOSYLATED HAEMOGLOBIN (HbA1C) Method:HPLC	5.4	%	Non-diabetic: <= 5.8 % Pre-diabetic: 5.8 - 6.5 % Diabetic: >= 6.5 %
* Approximate mean plasma glucose Method:Calculated	108.28	mg/dL	<111

INTERPRETATION::

A hemoglobin A1c (HbA1c) test measures the amount of blood sugar (glucose) attached to hemoglobin. If HbA1c levels are high, it may be a sign of diabetes, a chronic condition that can cause serious health problems, including heart disease, kidney disease, and nerve damage. Note: The HbA1c test is not used for gestational diabetes, a type of diabetes that only affects pregnant women, or for diagnosing diabetes in children. References American Diabetes Association. Standards of medical care in diabetes—2014. Diabetes Care. 2014 Jan;37 Suppl 1:S14-80.	HbA1C Value in %	Average Glucose in mg/dL	Impression
	4.0	68	Non-Diabetic
	4.5	82	
	5.0	97	
	5.5	111	
	6.0	125	Pre-Diabetic/Good Control
	6.5	140	
	7.0	154	Diabetic/Poor Control
	7.5	169	
	8.0	183	
	8.5	197	
	9.0	212	
	9.5	226	
	10.0	240	

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LAB MANAGERDr Arjumand Aleem MD
CONSULTANT
BIOCHEMIST,APMC/FMR/83361

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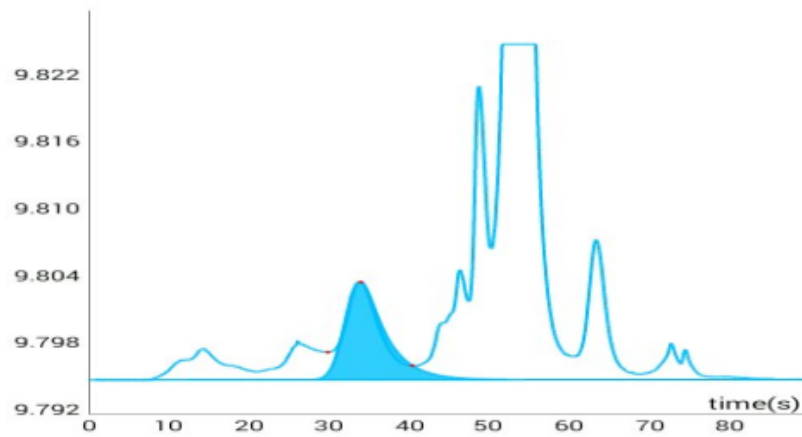
Result Report

Result Date : 2026-05-17 15:34:00

Test Code : 0114

Sample No : 3238252

Parameter	Value	Unit
HbA1a	0.4	
HbA1b	0.2	
HbF	1.4	
L-A1c	0.4	
HbA1c	5.4	
p3	3.7	
HbA0	83.6	
v-win	0.0	
eAG	108.0	





Name : Mrs. D SHARADHA 15786

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Sample Type : Serum

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Vial ID : 3238250

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Registered On : 17-May-2026 02:38 PM

Reported On : 17-May-2026 04:02 PM

Client Code : CMLTSD23

25-Hydroxy Vitamin D

Test Name	Observed Values	Units	Biological Reference Intervals
* 25-OH VITAMIN (VIT D3) Method: E C L I A	26.55	ng/mL	Deficiency: <10 Insufficiency: 10-30 Sufficiency: 30-100 Toxicity: >100

INTERPRETATION::

Vitamin D is a nutrient that is essential for healthy bones and teeth. There are two forms of vitamin D that are important for nutrition: vitamin D2 and vitamin D3. Vitamin D2 mainly comes from fortified foods like breakfast cereals, milk, and other dairy items. Vitamin D3 is made by your own body when you are exposed to sunlight. It is also found in some foods, including eggs and fatty fish, such as salmon, tuna, and mackerel.

Vitamin D deficiency is due to:

- Not getting enough exposure to sunlight
- Not getting enough vitamin D in your diet
- Having trouble absorbing vitamin D in your food

A low result may also be observed due to the body is having trouble using the vitamin as it should, and may indicate kidney or liver disease.

A vitamin D deficiency is usually treated with supplements and/or dietary changes.

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E-Glomerular Filtration Rate (EGFR)

Test Name	Observed Values	Units
* GLOMERULAR FILTRATION RATE (EGFR)	114	ml/min/1.73/m2

Method: Calculated

INTERPRETATION::

Biological Reference Ranges

Normal : >90 mL/min/1.73 m2

Mildly Decreased : 60-90 mL/min/1.73 m2

Mildly to Moderately Decreased: 45-59 mL/min/1.73 m2

Moderately to Severely Decreased: 30-44 mL/min/1.73m2

Severely Decreased: 15-29 mL/min/1.73m2

Kidney Failure: <15 mL/min/1.7m2

A normal eGFR for adults is greater than 90 mL/min/1.73m2, according to the National Kidney Foundation. (Because the calculation works best for estimating reduced kidney function, actual numbers are only reported once values are less than 60 mL/min/1.73m2). An eGFR below 60 mL/min/1.73m2 suggests that some kidney damage has occurred.

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Kidney Basic Screen

Test Name	Observed Values	Units	Biological Reference Intervals
* SERUM CREATININE Method:Jaffes	0.62	mg/dL	0.6 - 1.2
* UREA Method:Urease	21.3	mg/dL	Female <50yr:15-40 Female >50yr:21-43 Male <50yr:19-44 Male >50yr:18-55
* UREA NITROGEN (BUN) Method:Calculated	9.81	mg/dL	8 - 23
* BUN/CREATININE RATIO	15.83		
* URIC ACID Method:Uricase	2.33	mg/dL	Male:3.5 -7.2 Female:2.6 - 6.0
* SODIUM Method:ISE	141.3	mmol/L	136-145
* POTASSIUM Method:ISE	4.65	mmol/L	3.5 - 5.5
* CHLORIDE Method:ISE	100.3	mmol/L	95-105

INTERPRETATION::

kidney disease usually does not have signs or symptoms. Testing is the only way to know how your kidneys are doing. It is important to get checked for kidney disease if an individual having the key risk factors - diabetes, high blood pressure, heart disease, or a family history of kidney failure.

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Lipid Profile

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL CHOLESTEROL Method:Enzymatic	124	mg/dL	Desirable : < 200 Borderline: 200 - 239 High Risk : > 240
* HDL - CHOLESTEROL Method:Homogeneous enzymatic	45.3	mg/dL	Desirable Level :>59 Optimal :40-59 Undesirable :<40
* TRIGLYCERIDES Method:Enzymatic	88.2	mg/dL	Desirable Level :<150 Border line :150-199 High :200-499 Very High :>499
* VLDL CHOLESTEROL	17.64	mg/dL	≤ 30
* LDL - CHOLESTEROL	61.06	mg/dL	Optimal :<100 near Optimal :100-129 Borderline High:130-159 High :160-189 Very High :>189
* CHOL / HDL Ratio.	2.74	-	-
* HDL/LDL CHOLESTEROL RATIO	0.74	Ratio	-
* LDL / HDL RATIO	1.35	Ratio	Desirable level :0.5 3.0 Borderline Risk :3.0-6.0 High Risk :>6.0

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THYROID PANEL-I

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL TRIIODOTHYRONINE (T3) Method:ECLIA	1.18	ng/mL	0.82 - 2.0 Pregnancy: 1st Trimester: 0.81-1.90 2nd & 3rd Trimester: 1.0-2.6
* TOTAL THYROXINE (T4) Method:ECLIA	9.47	ug/dl	5.1 - 14.1 Pregnancy: 1st Trimester: 4.6-16.5 2nd & 3rd Trimester: 4.6-18.5
* Thyroid Stimulating Hormone.(TSH) Method:ECLIA	2.52	μIU/mL	Adult : 0.35 – 5.5 Pregnancy: 1 st trimester: 0.3- 4.5 2 nd trimester: 0.5- 4.6 3 rd trimester: 0.8- 5.2

INTREPRATATION:

The determination of T3 is utilized in the diagnosis of T3hyperthyroidism,the detection of early stages of hyperthyroidism and for indicating a diagnosis of thyrotoxicosis factitia.

The determination of T4 can be utilized for the following indications: the detection of hyperthyroidism, the detection of primary and secondary hypothyroidism, and the monitoring of TSH-suppression therapy.

The determination of TSH serves as the initial test in thyroid diagnostics.Even very slight changes in the concentrations of the free thyroid hormones bring about much greater opposite changes in the TSH level. Accordingly,TSH is a very sensitive and specific parameter for assessing thyroid function and is particularly suitable for early detection or exclusion of disorders in the central regulating circuit between the hypothalamus, pituitary and thyroid.

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LIVER FUNCTION TEST (LFT)

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL BILIRUBIN Method:Diazonium Ion	0.44	mg/dL	0.3 - 1.2
* DIRECT BILIRUBIN Method:Diazonium	0.14	mg/dL	0.0 - 0.2
* INDIRECT BILIRUBIN Method:Calculated	0.30	mg/dL	0.2 - 0.8
* SGOT / AST Method:IFCC without pyridoxaal phosphate	22.2	IU/L	Males: Upto 50 Females: Upto 35
* SGPT / ALT Method:Tris buffer without p5p	19.3	IU/L	Upto 33
* Alkaline Phosphatase(ALP) Method:PNP-AMP BUFFER,IFCC Rate analysis	71.2	U/L	35-104
* PROTEIN-TOTAL Method:Biurate	6.86	g/dL	6.4-8.3
* ALBUMIN Method:BCG	4.22	g/dL	3.57 - 5.42
* GLOBULIN Method:Calculated	2.64	g/dL	2.5 - 3.5
* A/G RATIO Method:Calculation	1.60	-	1.0 - 2.1
* GAMMA GLUTAMYL TRANSFERASE Method:Enzymatic colorimetric	26.3	U/L	Male : 8 - 61 Females : 5 - 36
* SGOT/SGPT RATIO	1.15		-

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Vitamin - B12

Test Name	Observed Values	Units	Biological Reference Intervals
* Vitamin - B12	501.3	pg/mL	211 - 946

Method: ECLIA

INTREPRATATION:

Vitamin B12 and folate are critical to normal DNA synthesis, which in turn affects erythrocyte maturation.3 Vitamin B12 is also necessary for myelin sheath formation and maintenance. The body uses its B12 stores very economically, reabsorbing vitamin B12 from the ileum and returning it to the liver so that very little is excreted. Clinical and laboratory findings for B12 deficiency include neurological abnormalities, decreased serum B12 levels, and increased excretion of methylmalonic acid. The impaired DNA synthesis associated with vitamin B12 deficiency causes macrocytic anemias. These anemias are characterized by abnormal maturation of erythrocyte precursors in the bone marrow, which results in the presence of megaloblasts and in decreased erythrocyte survival. Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to lack of intrinsic factor. Low vitamin B12 intake, gastrectomy, diseases of the small intestine, malabsorption, and trans-cobalamin deficiency can also cause vitamin B12 deficiency.

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Complete Blood Analysis (CBA)

Test Name	Observed Values	Units	Biological Reference Intervals
* HAEMOGLOBIN	9.5	gm%	12.0 - 15.0
Method:Photometry/Non-cyanmethhaemoglobin			
* RBC Count	3.52	millions/cumm	3.8-4.8
* WBC Count	6300	cells/cumm	4000-11000
* PLATELET COUNT	3.06	lakhs/cumm	1.5 - 4.5
Method:Impedence and Light Scattering			
* Packed Cell Volume(PCV)	28.2	%	40 - 50
Method:Cum.RBC Pulse Hight detection			
* MCV	80.0	fL	80 - 100
* MCH	26.9	pg	27 - 33
* M C H C	33.6	g/dL	32.0 - 34.0
* MPV (MEAN PLATELET VOLUME)	7.4	pg	7.5 - 11.2
* RDW - CV	15.6	%	11.5 – 14.5
DIFFERENTIAL COUNT			
* NEUTROPHILS,,	70	%	50 - 75
* LYMPHOCYTES	25	%	20 - 40
* EOSINOPHILS	02	%	02 - 06
* MONOCYTES	03	%	02 - 08
* BASOPHILS	00	%	0 - 1
* ABSOLUTE NEUTROPHIL COUNT	4410.00	cells/cumm	2000 - 7000
* ABSOLUTE LYMPHOCYTE COUNT	1575.00	cells/cumm	1000-3000
* ABSOLUTE EOSINOPHIL COUNT	126.00	cells/cumm	50-500
* ABSOLUTE MONOCYTE COUNT	189.00	cells/cumm	200 - 1000
* ABSOLUTE BASOPHIL COUNT	0.00	cells/cumm	00- 100
Method:Analyzer / Microscopy			
PERIPHERAL SMEAR			
* RBC	Normocytic Hypochromic		
* WBC	Total counts within normal limits. Differential cells study shows normal morphology and distribution.		
* PLATELETS	Adequate in number with normal morphology		
* ESR	18	mm/hr	0 - 10
Method:Westergren Method			



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MD PATHOLOGY,TSMC.NO: 90010

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Calcium			
Test Name	Observed Values	Units	Biological Reference Intervals
* CALCIUM	9.25	mg/dL	12–18 years: 8.4 – 10.2
Method:Photometric assay			18– 60 years: 8.6 – 10.0
			60– 90 years: 8.8 – 10.2
			> 90 years : 8.2 – 9.6

INTERPRETATION::

Calcium is one of the most important minerals in the body. Calcium needed for healthy bones and teeth. Calcium is also essential for proper functioning of the nerves, muscles, and heart.

Symptoms of high calcium levels include:

- Nausea and vomiting
- More frequent urination
- Increased thirst
- Constipation
- Abdominal pain
- Loss of appetite

Symptoms of low calcium levels include:

- Tingling in the lips, tongue, fingers, and feet
- Muscle cramps
- Muscle spasms
- Irregular heartbeat

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Iron Deficiency Profile

Test Name	Observed Values	Units	Biological Reference Intervals
* IRON Method:Ferron	19.1	µg/dL	33 - 193
* Total Iron Binding Capacity (TIBC) Method:Ferrozine	274	µg/dL	240 - 450
* TRANSFERRIN Method:Immunoturbidometry	186.39	ug/dl	176 - 280
* TRANSFERRIN SATURATION	6.97	%	20 - 50
* FERRITIN Method:Chemiluminescence	14.3	ng/mL	11.0 - 306.8

INTERPRETATION::

Iron is a mineral thats essential for making red blood cells. Red blood cells carry oxygen from the lungs to the rest of the body. Iron is also important for healthy muscles, bone marrow, and organ function. Iron levels that are too low or too high can cause serious health problems. iron levels that are too low include Pale skin, Fatigue, Weakness, Dizziness, Shortness of breath, Rapid heartbeat. iron levels that are too high include Joint pain, Abdominal pain, Lack of energy, Weight loss.

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Sample Type : Urine

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Complete Urine Analysis (CUE)

Test Name	Observed Values	Units	Biological Reference Intervals
PHYSICAL EXAMINATION			
* COLOUR	PALE YELLOW	-	Pale Yellow/Amber
* APPEARANCE	HAZY	-	Clear
* pH	5.0	-	5.0 - 9.0
Method:Double Indicator			
* SPECIFIC GRAVITY	1.025	-	1.005 - 1.030
Method:Ion Exchange			
CHEMICAL EXAMINATION			
* PROTEIN URINE	NEGATIVE	-	Negative
Method:Protein Error of Indicators/SSA			
* UROBILINOGEN	NEGATIVE	-	Negative
Method:Modified Ehrlich reaction			
* KETONE BODIES	NEGATIVE	-	Negative
Method:Nitroprusside Reaction			
* BILIRUBIN.	NEGATIVE	-	Negative
Method:Dip Stick/Hay's Sulphur			
* BLOOD	++	-	Negative
Method:Peroxidase like activity			
* NITRITE	NEGATIVE	-	Negative
Method:Diazotization/Griess test			
* GLUCOSE URINE	NEGATIVE	-	Negative
Method:GOD-POD/Benedicts test			
* LEUCOCYTES	+	-	Negative
Method:Leucocyte esterase			
MICROSCOPIC EXAMINATION			
* PUS CELLS.	6 - 8	/HPF	0-5
* EPITHELIAL CELLS	4 - 6	/HPF	0-5
* RBCS	8 - 10	-	NIL
* CASTS	Nil	-	Absent
* CRYSTALS	Nil	-	Absent
* BACTERIA	Nil	-	Absent
* OTHERS	Nil	-	NIL



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